Peri-Operative management of Direct Oral Anticoagulants (DOAC) in URGENT (not limb or life threatening) Surgery

Weigh patient and calculate Creatinine Clearance (CrCL) (nor estimated GFR)

Assess thrombotic and bleeding risks see guideline: YG CG 401

Pre-operative management of DOACs

CrCl (min/mL)	DABIGATRAN		RIVAROXIBAN & EDOXABAN		APIXABAN	
Bleeding risk	Standard risk and GA	High risk & Neuraxial Blocks	Standard risk and GA	High risk & Neuraxial Blocks	Standard risk and GA	High risk & Neuraxial Blocks
≥80	24 hrs	48 hrs	24hrs	48hrs	24 hrs	48 hrs
50-80	48 hrs	72 hrs				
30-49	72 hrs	96 hrs				
<30	Contraindicated		48 hrs	72hrs	48 hrs	72 hrs

Post –operative Management: when to restart DOAC post surgical intervention

Time of first dose of <u>any</u> DOAC after Neuraxial Procedure: 6 hours (24 hours if bloody tap)

Consider post operative risk of bleeding before re-starting.

All DOACs are rapidly absorbed and have a rapid onset of action, with peak anticoagulant activity at approximately 2-3 hours after oral ingestion. Attention to post-operative haemostasis is clinically important. Too early resumption of DOACs, especially within 24 hours of surgery, is associated with a two-to-fourfold increased risk of major bleeding.

Minor / Low Risk Procedures
Recommence 6 – 12 hours post-procedure if haemostasis has been fully secured.

High Risk Procedures / Increased Bleeding Risk
Do not recommence at full-dose until at least 48 hours post-procedure.

Consider prophylactic LMWH, commenced 6-12 hours post-op based on patient's thromboembolic risk and bleeding risk .LMWH should be discontinued immediately upon recommencing DOAC.

Reference:
Keeling D, Campbell Tait R, Watson H *et al* on behalf of the British Committee for Standards in Haematology. Peri-operative management of anticoagulation and antiplatelet therapy. British Journal of Haematology. 2016; 175:602-612